



Royal Perth Bentley Group: Mental Health Division

Safe Haven

Dabakan Ngowoort Koorliny Mia

Move to a calm, gentle, quiet
space

Model of Care and Service Plan

Traditional Owners Acknowledgement

The East Metropolitan Health Service recognises the Whadjuk People of the Noongar Nation as the traditional custodians of the land on which we learn and work today.

We acknowledge the Whadjuk people have a continuing spiritual and cultural connection to this land and we pay respects to Elders past, present and future.

Safe Haven Model of Care and Service Plan

Version: 2
Authorised by: Mental Health Medical Co-Director

24/02/2026

Date Compiled: 01/2021
Document Owner: Co-Director Mental Health Division

Revision Due: 02/2028

Method of Development

Consultation

Stakeholders from relevant departments and services at Royal Perth Bentley Group (RPBG) were engaged to provide subject matter expertise and advice for the development of this document. The Safe Haven Project Control Group (PCG) and Clinical Working Group were utilised to develop the service model and operational plans as outlined.

Disclaimer

The content of the Safe Haven Model of Care and Service Plan was developed by relevant stakeholders using their knowledge at the time. Details within the document may require modification as new information emerges.

Definitions

Attendee	Is the preferred term used for an individual who has been, is being, or will use a health service/ receive health care at Safe Haven and will be used throughout the document. An 'attendee' of Safe Haven meets the definition of "patient" in the Health Services Act 2016 (WA) (HS Act 2016) and may, depending if the person has a mental health concern, fall within the definition of "patient" in the Mental Health Act 2014 (WA) (MHA 2014). Safe Haven service and Attendees are therefore encapsulated in the legislation and regulations pertaining to both the HS Act (2016) and the WA MHA 2014. Attendees must be aged 18 years and above.
Clinician	Registered health professional employed by East Metropolitan Health Service (EMHS).
Peer Support Workers (PSWs)	Persons with a lived experience of mental health challenges who have accessed mental health services and are provided by RUAH Community Services under a contract arrangement with EMHS. PWs provide lived experience, psychoeducation and support service.

Safe Haven Model of Care and Service Plan

Version: 1
 Authorised by: Consistent High Quality Care Committee
 08/02/2021

Date Compiled: 01/2021
 Document Owner: Director Clinical Training MH Division
 Revision Due: 02/2024

Research Electronic Data Capture (RedCAP)	A Department of Health secure electronic data system to facilitate collection and analysis of Attendee details, including a screening tool with questions targeting several areas of brief screening: • Reasons for the person attending • Knowledge of the service and what they hope to be able to undertake or receive at Safe Haven Observational indicators will be undertaken to ensure that the persons needs can be accommodated.
Safe Haven team	Location N Block Royal Perth Hospital (RPH) on Victoria Square opposite St Mary's Cathedral, Perth Staff Composition • Peer Support Workers (PSW) - two [2] • Senior Clinician (one [1] – Allied Health) • 0.1FTE Nurse Unit Manager (NUM) - managerial support Hours of Operation • Safe Haven team working hours are 15:00 to 20:00 hours on Friday – Sunday. Excludes public holidays • NUM hours rostered 1day/fortnightly - TBA
Peer Support	A system of giving and receiving help that is founded on the key principles of respect, shared experience and responsibility, and mutual agreement. When people find affiliation with others they feel they are 'like' them, they feel a connection. This connection, or affiliation, is a deep, holistic understanding based on mutual experience without the constraints of traditional (expert/patient) relationships¹. Peer support is not based on psychiatric models and diagnostic criteria.

Safe Haven Model of Care and Service Plan

Version: 1
 Authorised by: Consistent High Quality Care Committee
 08/02/2021

Date Compiled: 01/2021
 Document Owner: Director Clinical Training MH Division
 Revision Due: 02/2024

CONTENTS

Method of Development	2
Consultation	2
Disclaimer	2
Definitions	2
CONTENTS	4
1.0 Overview	5
1.1 Purpose	5
1.2 Strategic Alignment	5
1.3 Key Service Principles	5
1.4 Model of Care Objectives	6
1.5 Service Delivery Profile	6
1.5.1 Pathways	6
1.5.2 Criteria for Attendance	6
1.6 Service Provision	7
2.0 Attendee Post Satisfaction	7
3.0 Governance Structure	8
4.0 Workforce	8
4.1.1 Team composition	8
4.1.2 Peer Support Worker Qualifications and Training	10
5.0 Facility Management	11
5.1.1 N Block Room Booking System Procedure	11
5.1.2 Facility Design	12
5.1.3 Swipe Keycard Access	12
5.1.4 Closed Circuit Television	12
5.1.5 Hardwired Duress System	12
6.0 Non- Clinical Support Services	12
6.1.1 Cleaning	12
6.1.2 Catering	12
6.1.3 Linen Management	13
7.0 Operational Performance and Evaluation	13
7.1.1 Safe Haven KPIs	13
8.0 Related Policies/Documents	14
8.1.1 Policies and Documents	14
Appendix I: Collaborative Recovery Peer Support Work Models	15
Appendix 2 : Safe Haven Outcome Measures	17

Safe Haven Model of Care and Service Plan

Version: **1**
 Authorised by: **Consistent High Quality Care Committee**
08/02/2021

Date Compiled: **01/2021**
 Document Owner: **Director Clinical Training MH Division**
 Revision Due: **02/2024**

1.0 Overview

For many people experiencing crisis, emergency departments (ED) may be accessed and form the only support they may have. However, a busy ED is not ideal for people who are in psychological distress. An alternative option to ED to better support individual needs and help vulnerable people access the appropriate care and foster connectedness is essential.

The RPBG Mental Health Division has established an alternative service to ED able to provide psycho-social support to individuals experiencing psychological distress within the Perth metropolitan area – Safe Haven.

1.1 Purpose

The purpose of this Service Plan is to outline the following:

- Model of care
- Service delivery
- Operational workflows
- Staffing requirements
- Governance and management
- Key Performance Indicators (KPIs).

1.2 Strategic Alignment

The Safe Haven service strategically aligns with several government and Department of Health initiatives, including:

- Western Australia (WA) Sustainable Health Review (SHR), Strategy 2, Recommendation 7
- Western Australian Mental Health, Alcohol and Other Drug Services Plan 2015-2025 Update 2018
- RPBG Mental Health Emergency Centre project.

Safe Haven aligns with the RPH SERVIO philosophy and EMHS Strategic Intent.

1.3 Key Service Principles

The key service principles that guide service provision:

- The six (6) Peer competencies.¹
- Aspects of the Sanctuary model, Safe Wards and the Collaborative Recovery Model. For further information, see Appendix 1.

Safe Haven Model of Care and Service Plan

Version: 1
 Authorised by: Consistent High Quality Care Committee
 08/02/2021

Date Compiled: 01/2021
 Document Owner: Director Clinical Training MH Division
 Revision Due: 02/2024

1.4 Model of Care Objectives

The key objectives of Safe Haven service delivery include:

Benefit Description	Estimated Benefit
Reduction in the number of Triage category 4 or 5 presentations during the hours Safe Haven is open	Reduced number of attendances or reduction in the ED length of episode for mental health patients who are suitable for Safe Haven as an alternative to ED.
Improved experience of Service for Attendees utilising Safe Haven as an alternative to ED	- Provision of an alternative to ED attendance after-hours that is PW identified and led to provide opportunities for learning or reinforcing the use of self-management strategies and the establishment of social connections - Provide a suitable environment and model of lived experience PWs who deliver de-escalation, situation distress management and emotional regulation strategies on an individual/ group basis - Provision of information/referral to services and guided self-management strategies to Attendees to assist with managing crises.

1.5 Service Delivery Profile

1.5.1 Pathways

Safe Haven accepts Attendees via the following pathways:

- ED diversion i.e. RPH ED
 - If a person is triaged in ED /ED wait room the ED Clinician may direct the person to Safe Haven
 - ED Concierge/Volunteer to provide directions to Attendee regarding Safe Haven location, as required
 - Self-presentation, i.e. based on a community recommendation (e.g. community mental health services [CMHS], hospitals/ health services, Non-Government Organisations [NGOs]).

1.5.2 Criteria for Attendance

Safe Haven is suitable for Attendees aged 18 years+ experiencing distress/crisis.

For further information relating to criteria for attendance, refer to the **RPHG Safe Haven: Access and the Provision of Care Standard Operational Procedure (SOP)**.

Safe Haven Model of Care and Service Plan

Version: 1
 Authorised by: Consistent High Quality Care Committee
 08/02/2021

Date Compiled: 01/2021
 Document Owner: Director Clinical Training MH Division
 Revision Due: 02/2024

1.6 Service Provision

Underpinned by peer support work model, Safe Haven provides Attendees:

- An alternative to present to ED
- Conveys hope, mutual support and connectedness in a safe space to promote recovery through sharing lived experiences
- Validates experiences and feelings
- Encourages personal efforts to enhance health and wellness
- Encourages and supports personal recovery practices
- Encourages personal exploration and pursuit of community roles
- Validation of efforts and accomplishments
- Mindfulness, brief intervention, stress reduction strategies, interpersonal effectiveness and emotional regulation skills
- Access to self-regulating activities may be individual or group based (i.e. yoga, music, drawing, art, etc.)
- Access to available services i.e. shower facilities
- Support plan development (i.e. wellness plan) that includes use of the wider network of health and care services, where relevant
- Practical assistance to accomplish tasks and goals
- Offers small refreshments.

For further procedural details, refer to the **RPBG Safe Haven: Access and the Provision of Care SOP**.

2.0 Attendee Post Satisfaction

An Attendee may choose to provide feedback about Safe Haven services in the following ways:

- Electronic Safe Haven Postcard
- EMHS electronic satisfaction survey
- Verbal in-person feedback
- EMHS/RPBG Feedback forms available onsite
- Care Opinion

Further information: <https://rph.health.wa.gov.au/About-us/Compliments-and-Complaints>

Safe Haven Model of Care and Service Plan

Version: 1
 Authorised by: Consistent High Quality Care Committee
 08/02/2021

Date Compiled: 01/2021
 Document Owner: Director Clinical Training MH Division
 Revision Due: 02/2024

3.0 Governance Structure

The governance of Safe Haven sits with the RPBG Mental Health Division.

When an Attendee accesses Safe Haven, they will not be considered 'admitted'. An attendee is someone who is owed a duty of care by staff of the Safe Haven (health professionals or not).

4.0 Workforce

4.1.1 Team composition

Safe Haven team member roles and responsibilities are outlined as follows.

Nurse Unit Manager (NUM)

0.1 FTE NUM for managerial support.

The NUM provides the single point of accountability for clinical and management leadership to all staff within the service, including

- Provides leadership to nursing, medical, allied health professionals and PWs
- services within and external to the EMHS
- Completion of contractual requirements, ordering of stock, FTE, managing Datix Clinical Incident Management System (CIMS) and/or Combined Hazard or Injury Report (CHoIR), as required
- Facilitates the provision of advanced and complex patient care within the area of speciality at a health service level working in a leadership role with the interdisciplinary team members and external providers
- Ensures as far as practicable, the provision of a safe working environment in consultation with staff under their supervision
- Initiates and participates in the development of quality improvement and risk management strategies for the service and Service wide
- Provides an expert consultancy service for a broad range of stakeholders, internal and external to the health service.

Safe Haven Model of Care and Service Plan

Version: 1
 Authorised by: Consistent High Quality Care Committee
 08/02/2021

Date Compiled: 01/2021
 Document Owner: Director Clinical Training MH Division
 Revision Due: 02/2024

Two Co-reflection and Group Peer Support Workers

A Peer Support Worker is employed to assist Attendees, carers and families, in the provision of support for Attendees to develop a better understanding of the recovery framework while utilising a lived experience of mental health challenges and to work towards achieving the best outcomes for Attendees.

Key accountabilities of the role are:

- Provides direct support and interaction to Attendees under the direction of the Senior PW in a manner consistent with recovery philosophies including but not limited to:
 - Providing a welcoming reception and interaction to Attendees and carers
 - Assisting Attendees to identify their own strengths and goals to empower positive change
 - Supporting and encouraging Attendees to access activities that aim to enhance their recovery
- Serve as a positive role model to Attendees and/or staff as a source of information on personal experience of managing mental health illness/issues

Safe Haven Model of Care and Service Plan

Version: **1**

Authorised by: **Consistent High Quality Care Committee**

Date Compiled: **01/2021**

Document Owner: **Director Clinical Training MH Division**

- Provision of general feedback to the team regarding Attendee and/or carer participation, learnings, support and recovery-oriented practice
- Promotion of physical health and other areas of wellbeing that support recovery
- Performing administrative tasks including replenishing materials and resources, sending and receiving emails, record keeping, collection of Attendee and carer feedback
- Actively participating in and providing peer support worker led groups
- Facilitation and planning of groups
- Supporting Attendees to make positive changes towards recovery by identifying strengths and goals and problem-solving strategies, including
 - Support plan development (i.e. wellness plan) that includes use of the wider network of health and care services, where relevant
- Assist Attendees in their recovery planning from a peer perspective.

4.1.2 Peer Support Worker Qualifications and Training

Peer Support Workers will be trained in the following:

- Self-care and self-management skills training (working in a very triggering environment)
- Emotional competence
- Coaching and advocacy training
- Tertiary completion of Mental Health Peer Work Certificate IV or progression towards completion
- Suicide prevention (Safe talk and Asist)
- Managing conflict
- Rainbow tick – LGBTI+ Training
- De-escalation and ACE breakaway techniques
- Understanding Personal Boundaries
- Hand Hygiene
- Aboriginal Cultural Competency
- Working with families and carers.

Clinician

The key responsibilities of this role are to

- Undertake screening of Attendees to ensure that their needs are of a non-clinical nature consistent with the service type
- Provision of group and individual based interventions in collaboration with PWs
- Provision of a coaching and mentorship role to PWs should clinical escalation or redirection to facilitate care be required
- Provision of assessment and management of crisis interventions at an advanced level with experience in the health system, peer support work and referral pathways
- Complete all documentation requirements (at entry and exit) including entering information into RedCAP
- Where escalation of care is required for an Attendee, follow processes outlined in the 'Escalation of care' section of the **RPBG Safe Haven: Access and Provision of Care SOP**.

Volunteer workforce

A future goal post opening of Safe Haven is the establishment of a volunteer workforce to provide PW opportunities in accordance with the **EMHS Volunteer Policy**. The volunteer model will focus on opportunities for individuals undergoing tertiary education in peer support work (or similar) who require placement or for individuals interested in participating in volunteer work in the mental health space.

Fulfill all requirements specified in the **EMHS Volunteer Policy**, including relevant reference checks and orientation and training in accord with the **RPH Emergency Procedures** and those specific to Safe Haven.

5.0 Facility Management

5.1.1 N Block Room Booking System Procedure

The N Block room booking system is managed by the RPBG Executive Office Manager. Room availability and bookings can be made by contacting (x42856).

Safe Haven has a scheduled booking for Friday to Sunday 1430-2000hrs.

Safe Haven Model of Care and Service Plan

Version: 1
 Authorised by: Consistent High Quality Care Committee
 08/02/2021

Date Compiled: 01/2021
 Document Owner: Director Clinical Training MH Division
 Revision Due: 02/2024

5.1.2 Facility Design

Safe Haven is located in a single storey facility that has two steps on entry and disability access via a ramp at the rear including:

- Video Intercom technology for entry
- Swipe card access
- Four bathrooms (including disabled toilet)
- Shower (located in the disabled toilet)
- Hardwire duress system
- Single landline telephone
- Air conditioning
- Closed Circuit Television (CCTV) at entry and exit
- Universal Serial Bus (USB) ports for charging of phone, tablet and laptop
- Refrigerator, coffee machine and microwave
- Lockers for attendee belongings
- Soft and hard furnishings.

5.1.3 Swipe Keycard Access

An electronic lock management system is installed to enable staff ID swipe card access to the front and internal entry door to Safe Haven. Issuing access to Safe Haven is facilitated via Security as per **EMHS Security: Access Control Policy**.

5.1.4 Closed Circuit Television

CCTV is linked to the existing RPBG security system with a monitor located within Safe Haven to allow surveillance of the entry/exit doors. For further guidance, an outline is found in the **RPH Emergency Procedures (Safe Haven)/ RPBG Safe Haven: Access and Provision of Care SOP**.

5.1.5 Hardwired Duress System

For information about the duress alarms linked to the existing RPBG Security system refer to the **RPBG Safe Haven: Access and Provision of Care SOP**.

6.0 Non-Clinical Support Services

6.1.1 Cleaning

The cleaning of Safe Haven (N Block) is scheduled to occur every Monday morning 0700-0900 by RPBG Patient Support Services (PSS). Staff are to contact PSS for any additional cleaning requirements and for further information on cleaning duties see PSS Cleaning Manual.

6.1.2 Catering

Light refreshments will be provided by RPBG PSS (Catering).

Safe Haven Model of Care and Service Plan

Version: 1
 Authorised by: Consistent High Quality Care Committee
 08/02/2021

Date Compiled: 01/2021
 Document Owner: Director Clinical Training MH Division
 Revision Due: 02/2024

6.1.3 Linen Management

For information relating to linen management for Safe Haven refer to the **RPBG Safe Haven: Access and the Provision of Care SOP**.

7.0 Operational Performance and Evaluation

The Safe Haven Team is responsible for the collection and reporting of KPIs to measure benefits and enable agile decision making to improve service delivery. Safe Haven will comprise of outcome measures specific to the EMHS-Mental Health Commission (MHC) Commission Service Agreement (CSA) and measures of individual outcome, see Appendix 2: Safe Haven Outcome Measures.

RPBG Mental Health Division Co-Directors are responsible to ensure that data is collected in an appropriate, secure manner and that reporting against KPIs occurs as per the MHC CSA at six monthly intervals.

7.1.1 Safe Haven KPIs

KPI	Measure	Data Source
Attendance numbers by Day of Week/Time of Day	- Count of numbers attending - Count of repeat Attendees - Count of suitable attendances - Count of redirected attendances	RedCap collection screening tool
Referral Source count	Count of: - ED - Community referral - Self referral	RedCap survey collection
Reduction in ED Mental Health attendances (cat 4 and 5)	Count of Mental Health Triage Category 4 and 5, discharge disposition during Safe Haven opening hours day of week, time of day	Emergency Department Information System (EDIS), Safe Haven Dashboard display
Attendee Satisfaction	Subjective unit of distress (SUDs) ratings entry and exit and index of change	Self Assessment Tool in RedCAP
	Reported satisfaction with the service	Care Opinion Safe Haven Postcard EMHS Feedback forms

Safe Haven Model of Care and Service Plan

Version: 1
 Authorised by: Consistent High Quality Care Committee
 08/02/2021

Date Compiled: 01/2021
 Document Owner: Director Clinical Training MH Division
 Revision Due: 02/2024

KPI	Measure	Data Source
Attendee profile	Count of reported reasons for attending Safe Haven	RedCap survey collection
Clinical Incidents	Count of Clinical Incidents and rating	Datix Clinical Incident Management System (CIMS)

8.0 Related Policies/Documents

8.1.1 Policies and Documents

This Service Plan should be read in conjunction with the following policies and documents:

- RPBG
 - Safe Haven: Access and the Provision of Care SOP
 - Clinical Care for People Who May Be Suicidal Policy
 - RPH Emergency Department: Triage and Waiting Room Triage SOP
 - RPH Paediatric Presentations in the Emergency Department SOP
- EMHS Prevention and Management of Workplace Aggression and Violence Policy
- WA DOH Sustainable Health Review Final Report
- WA State Priorities Mental Health, Alcohol and Other Drugs 2020-2024 plan
- WA Mental Health Policy Framework:
 - Clinical Care of People Who May Be Suicidal Policy – MP0074/17
 - Clinical Care of People With Mental Health Problems Who May be At Risk of Becoming Violent or Aggressive Policy – MP0101/18
 - Community Mental Health Status Assessments Role of Mental Health Clinicians – MP0099/18
 - Statewide Standardised Clinical Documentation (SSCD) for Mental Health Services – OD 0526/14.

Safe Haven Model of Care and Service Plan

Version: 1
 Authorised by: Consistent High Quality Care Committee
 08/02/2021

Date Compiled: 01/2021
 Document Owner: Director Clinical Training MH Division
 Revision Due: 02/2024

Appendix I: Collaborative Recovery Peer Work Models

Aspects of the Sanctuary model, Safe Wards and the Collaborative Recovery Model (NEAMI, community based organisation) have been incorporated to ensure that appropriate language is used while still focusing on the aspects of adjustment that pose the greatest problems for any treatment environment ^{2, 3, 4}.

The Sanctuary Model is structured around a philosophy of belief and practice referred to as the **Sanctuary Commitments** (non-violence, emotional intelligence, social learning, open communication, democracy, social responsibility, growth and change) and the **S.E.L.F domains**.

The four key domains of healing are:

<i>Safety:</i>	attaining safety in self, relationships, and environment.
<i>Emotional</i>	identifying levels of various emotions and modulating emotion in response to memories, persons, events.
<i>Loss:</i>	feeling grief and dealing with personal losses and recognizing that all change involves loss; and
<i>Future:</i>	trying out new roles, ways of relating and behaving as a “survivor” to ensure personal safety and help others.

Safewards strategies applicable for Safe Haven

Safewards is a model designed to improve the safety for everyone including mental health service staff, consumers and visitors.ⁱⁱ The model has several intervention strategies, including those outlined below applicable to the Safe Haven model.

Clear mutual expectations

These expectations work both ways; just as the staff have expectations of consumers, consumers have expectations of the staff. As part of the codesign process mutual expected behaviours have been outlined with posters and displays constructed to communicate these expectations.

Safe Haven Model of Care and Service Plan

Version: 1
 Authorised by: Consistent High Quality Care Committee
 08/02/2021

Date Compiled: 01/2021
 Document Owner: Director Clinical Training MH Division
 Revision Due: 02/2024

Calm down methods

Sometimes we can recognise early signs of agitation. It might be someone's facial expression, tone of voice, quick response to a common reminder, restlessness, change of breathing pattern, body language, eye contact (or lack thereof), movement around the ward or other cues to use the person's own strengths and usual coping mechanisms to help them calm down.

- hot or cold scented towels
- relaxing music (iPods with classical, chill out and new age relaxation music)
- stress balls
- massage balls
- personal fan
- herbal tea
- blankets with different thicknesses and textures
- cold drink/hot drink/milky drink.

Soft words and Reassurance

This approach portrays and expresses genuine respect, empathy and warmth for a person as an equal by demonstrating understanding of how frustrating or difficult a situation or experience may be. The approach focusses on whole body listening and ensuring that one's body language and power position is considered at all times. Reassurance is the action of removing someone's doubts or fears and providing recognition or affirmation that a person or their feelings or opinions are valid or understood.

The (NEAMI) Collaborative Recovery Model⁴

Recovery is a way of describing the personal journey that takes place for people with mental health problems as they work toward a renewed sense of identity, meaning and purpose. Embedded in this concept is the belief that this is a unique journey for each person and therefore it needs to be a self-directed process.

The Collaborative Recovery Model uses protocols to assist people to identify their individual journey of recovery:

- To bring into focus important values and strengths – identifying what is important to the individual
- To identify an individual's longer-term vision or direction for their recovery journey and to track progress towards achieving identified goals
- To plan the next steps, anticipating any barriers and identifying the people and things that can assist along the way.

Safe Haven Model of Care and Service Plan

Version: 1
 Authorised by: Consistent High Quality Care Committee
 08/02/2021

Date Compiled: 01/2021
 Document Owner: Director Clinical Training MH Division
 Revision Due: 02/2024

Appendix 2 : Safe Haven Outcome Measures

Objectives		
1) Provide a peer based, non-clinical alternative to ED for people with mental health, alcohol and other drug issues, requiring assistance and support after hours, but not requiring intensive clinical support, when other support services are closed.		
2) Provide a safe, therapeutic but non-clinical space that supports and empowers people experiencing mental health/alcohol and other drug issues		
OUTCOME Indicators		
Indicator	Information required	Information Source
Positive impacts of the service	Number of clients who indicated 'alternative to ED' as a key positive impact from accessing the Safe Haven café	Staff or Consumer rated
	Number of clients who indicated 'referral to appropriate services' as a key positive impact from accessing the Safe Haven café	
	Number of clients who indicated 'de-escalation of distress' as a key positive impact from accessing the Safe Haven café	
	Number of clients who indicated 'interaction with peerworkers' as a key positive impact from accessing the Safe Haven café	
	Number of clients who indicated 'accessible and inclusive' as a key positive impact from accessing the Safe Haven café	
	Number of clients who indicated 'de-escalation of distress' as a key positive impact from accessing the Safe Haven café	
Output Indicators		
Indicator	Information required	Information Source
Service utilisation	Number of attendees	Staff collected
	Referral source: - referred from ED - self-referral - referred from other services	Staff collected
	Number of repeat attendees - 'Is this your first time accessing the Safe Haven Café?'	Staff or Consumer rated
Client profile - Reason for accessing the Safe Haven Café	Number of attendees who reported, as a reason for accessing the Cafe: - 'alternative to ED' - 'No other out-of-hours services, except ED' - 'MH issues' - 'AOD issues' - 'Domestic violence' - 'rough sleeping' - 'social interaction'	Staff collected
Demand for the service	To be decided	Staff collected
**This Interim Evaluation Framework is likely to change depending on feedback from relevant agencies and Treasury.		

Safe Haven Model of Care and Service Plan

Version: 1

Authorised by: Consistent High Quality Care Committee
08/02/2021

Date Compiled: 01/2021

Document Owner: Director Clinical Training MH Division

Revision Due: 02/2024

- ¹ Guiding Principles for Lived Experience (Peer) work, 2024, available from: https://www.mhc.wa.gov.au/media/4911/mhc-lived_experience_workforce-framework-mar2024-digital.pdf
- ² Miller, J.B. & Stiver, I. 1997. The Healing Connection. Boston, MA: Beacon Press.
- ³ SafeWards Victoria, 2016. Safewards Handbook. Training and implementation resource for Safewards in Victoria, State of Victoria, Department of Health and Human Services, available at: <https://www2.health.vic.gov.au/-/media/health/files/collections/policies-and-guidelines/s/safewards-victoria-handbook-2016.pdf>
- ⁴Neami National, Australia. W: <https://www.neaminational.org.au>

Document History

Version	Date	Author	Description
0.1	16/11/2020	Kirsty Snelgrove	First draft
0.2	04/12/2020	Michelle De Silva/Marika Spaseska	Coordinator of MH Nursing
0.3	14/12/2020	Michelle De Silva/Marika Spaseska	Coordinator of MH Nursing/Project Control Group
0.4	22/12/2020	Michelle De Silva/Marika Spaseska	Coordinator of MH Nursing/Project Control Group
0.5	24/12/2020	Michelle De Silva/Marika Spaseska	MH Division Co-Director

Document Endorsement

Position/Title	Name	Signature	Date
MH Division Co-Director	Wade Emmeluth		24/12/2020
Executive Director, Clinical Service Strategy & Population Health, EMHS	Joel Gurr		24/12/2020

Document Approval

I, the undersigned, have approved this document.

Position/Title	Name	Signature	Date
Nursing Director Organisational Learning & Development, RPBG	Heather Kidd		09/02/2021

Safe Haven Model of Care and Service Plan

Version: **1**
 Authorised by: **Consistent High Quality Care Committee**

Date Compiled: **01/2021**
 Document Owner: **Director Clinical Training MH Division**

08/02/2021

Revision Due: **02/2024**